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'I'm not trained, I'm not qualified': a systematic review of the mental health literacy amongst British university academics

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ABSTRACT

For most university students, their tutors or lecturers play an important role when they are experiencing mental health problems (MHPs), and these academics have a responsibility to support students during this. This review aimed to examine the current mental health literacy (MHL) amongst university academics. A systematic review of studies that explored the MHL of academics at UK universities was conducted. It followed the PRISMA guideline and is registered on PROSPERO. Searches were undertaken on six databases, accessed through EBSCO. The studies were limited to peer-reviewed, academic journals, published from 2014 onwards and in English. A further search of reference lists and related citations was conducted. The studies were assessed using the Critical Appraisal Skills Programme Checklist and the Mixed Methods Appraisal Tool. A thematic analysis was conducted following Braun and Clarke's six steps. Six studies, including qualitative and mixed methods were included. Four main themes were identified: the academics' ability to recognise MHPs, the academics' ability to manage MHPs, support that the university provides to academics and adverse effects experienced by the academics from supporting students. This review has found that there is a lack of support from universities for their academics in managing students with MHPs through inadequate training and guidance, due to national policies. Therefore, academics struggle to recognise and manage MHPs in students while experiencing difficulties in managing their own workload and wellbeing. This review recommends that universities should increase their support for academics through training, guidance and accessible support systems.

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mental health problems;
British university; British
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1. Introduction

The term 'mental health literacy' was first introduced in 1997 by Jorm and colleagues and is defined as the 'knowledge and beliefs about mental health disorders which aid their recognition, management or prevention' (Jorm et al. 1997). It is further referred to as 'the ability to recognise specific disorders; knowing how to seek mental health information; knowledge of risk factors and causes, of self-treatments, and of professional help

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available; and attitudes that promote recognition and appropriate help-seeking' (Jorm et al. 1997). Mental health literacy (MHL) is vital amongst university academics; as G. Hughes et al. (2018) states, academics are often the first point of contact when students are facing difficulties because they have a pre-existing relationship, are accessible and approachable. In this sense, MHL can also be understood to be the support that academics provide to their students, whether academics can recognise, are aware of or have had training on mental health problems (MHPs).

Universities can be a place where students experience pressure on their mental health (Akinlotan and Wagstaff 2025). More academics are encountering students with MHPs, as Spear et al. (2020), revealed 96% of academics encountered students with MHPs. In a study conducted in 2022 by the mental health charity, Student Minds, 57% of respondents who were students had self-reported a mental health issue; of this percentage, 24% have reported having been diagnosed with a mental health condition (Student Minds 2022). Another study conducted by the National Union of Students on behalf of the All Party Parliamentary Group on Students (2015) revealed that 33% of them had experienced suicidal thoughts in the past academic year. This is worrying as mental health conditions are worsening in the UK. In only a few years, between 2016/17 to 2022/23, the number of undergraduate students at universities across the UK who said they have experienced MHPs rose from 6% to 16% (Sanders 2023). There are many reasons for the increase in mental health conditions including being away from home, academic pressure, financial pressure, social pressure and the pressure of making friends (Guzder 2024). McManus and Gunnell (2020), however, provided a contrasting view in that it is unclear whether students at university are experiencing more MHPs than their non-student peers. Despite this, academics play a critical role in supporting students with MHPs, and the growing mental health needs present in the student population in the UK indicate that conducting a systematic review of MHL among academics is both timely and essential.

While there is uncertainty about whether universities have a 'duty of care' towards their students according to the Parliamentary Research Briefing (2021), there has been pressure to place a legal responsibility upon universities regarding this (Universities UK 2023). This can be interpreted as 'not to cause harm by careless acts or omissions' and as 'further legal duties relating to contract, health and safety, human rights, the safeguarding of vulnerable adults and equalities legislation' (Universities UK 2023). Universities often practice this by assigning an academic supervisor or tutor to support students. Many of these academics have reported seeing an increase in the number of students experiencing mental health difficulties, as compared to the past (G. Hughes et al. 2018). Academics have stressed their importance as a 'first point of contact' for students (Bennett et al. 2024). This highlights the need for academics to uphold a level of MHL in order to recognise and appropriately manage MHPs in their students.

Bronfenbrenner's Ecological systems Theory (Bronfenbrenner 1979) can help to understand the various factors that can impact an academic's MHL. According to McLaren and Hawe (2024), an ecological perspective can demonstrate the individual and contextual systems and how the two are interdependent. The theory is particularly useful to understand mental health as MHPs involve processes that act at different levels; it allows us to view mental health from biological, psychological and sociocultural perspectives (Kendler 2008). By implementing this theory, the various factors affecting MHL can be made evident together with how they are interconnected. At the microsystem, the academic's

own personal attitudes and experiences will be affecting their approach. Next, the mesosystem shows how processes at the microsystem interact with each other. For example, academics' personal views can impact their relationships and interactions with each other, which in turn will affect MHL outcomes -- this can manifest in the workplace culture and the support they provide to each other. The ecosystem involves factors such as training offered to academics, their workload, policies and guidance offered by the university and whether there is funding available for mental health support (McLinden 2016). Next, at the macrosystem, societal attitudes towards mental health problems, national campaigns and policies, cultural ideas and stigma can impact MHL. Finally, at the chronosystem, the changes over time affecting MHL can be revealed, the most pertinent example being COVID-19 and how that has impacted not just MHPs in students but how MHL can be supported by academics in a more digitalised and remote learning environment (Idris et al. 2021).

So far, there has been no other systematic review conducted which explores the MHL of academics at British universities. Although there have been other similar systematic reviews that have been conducted including that by Kristin, Gupta, and Keating (2018) and Suwanwong et al. (2024), none of these systematic reviews are specific to academics at UK universities supporting MHPs in students. This gap draws attention to the need to conduct a systematic review in order to investigate this further in a UK-specific context. The choice of utilising a systematic review for this research is based on the understanding that it will allow the synthesis of available data in a structured and transparent way to answer the research question and inform evidence-based recommendations.

1.1. Objective

The objective of this review is to undertake a systematic review of primary studies to explore MHL amongst academics at British universities. The research question for this systematic review is: 'what is the MHL amongst academics at British universities?'.

2. Methodology

2.1. Protocol and registration

This review registered under the PROSPERO protocol on the 11 June 2024, (CRD42024541653) and follows guidelines set out by the PRISMA statement (Moher et al. 2009).

2.2. Search strategy

An initial search was conducted on 19 June 2024, through the following sources: APA PsycInfo, British Education Index, CINAHL Plus with Full Text, ERIC, MEDLINE, Psychology and Behavioural Sciences Collection. These were all accessed through EBSCO. A wide range of search terms were utilised to access as many articles as possible that conducted primary research into the MHL of British academics. The search terms included:

- Mental health literacy, understanding, training, support, recognition, counselling, knowledge, insight, awareness, education, competence.
- Academic(s), lecturer(s), professor(s), university teacher(s), faculty(ies), educator(s).
- UK, United Kingdom, Britain, Great Britain, British Isles, England, Scotland, Wales, Northern Ireland.
- University(ies), postgraduate(s), graduate(s), higher education, college, master's, doctoral, higher learning institution(s), institution(s), academy(ies).

Appendix A demonstrates the search strategy. These search terms were based on synonyms of MHL and the understanding of mental health issues. The filters that were added included limiting the search to being from 2014 and onwards and in the English language. Limiting the studies to the last 10 years allows for the recommendations to be relevant and specific to the current context. Furthermore, this review is based in the UK, this is due to the increasing awareness of MHPs at British universities and so that the findings and policy recommendations can be sensitive and specific to the UK's sociocultural environment.

2.3. Inclusion/exclusion criteria

The studies that could be included in this systematic review include those that explore British academics' MHL. The term mental health literacy was operationalised to mean the 'understanding' or 'support' provided for MHPs. For example, during the search 'understanding' could mean how a MHP is recognised or perceived or the awareness surrounding it, while 'support' could mean the interventions put forward to address that MHP. The term 'British academic' was operationalised in the search to mean any individual that held a teaching role of any subject within a British university and came into contact with university students. This was done by reading the text and abstract, and occasionally the full text. The inclusion criteria include studies that utilise primary research through either quantitative, qualitative or mixed-methods. The studies were peer-reviewed, journal articles, from the UK (England, Scotland, Wales and Northern Ireland) written in English and published from 2014 onwards. Please refer to [Table 1](#).

2.4. Selection process and data collection

During the initial search 9816 studies were identified after duplicates were removed. The studies were then screened by their title and abstract. Studies were then excluded at this stage if they didn't relate to an academic setting or didn't explore mental health problems or didn't relate to academics or anyone with a teaching role at a university as apparent from the title and abstract. The studies were then reduced from 9816 to 182. After this, full text screening was conducted. Studies were excluded at this stage if they were not exploring UK universities, or were considering UK universities along with foreign universities without presenting them separately. Some studies were also excluded if they investigated teachers or academics outside of a university (for example a school) or explored academics in universities and schools without presenting the information

separately. They were further excluded if studies were looking at ‘disabilities’ as a whole and not separately exploring mental health problems.

To ensure rigour, increase quality and minimise bias, all the steps involved in the search strategy including defining the search terms and eligibility criteria, screening of results and selection of the final studies were independently completed by the two reviewers (authors). After the initial search by the first reviewer, the second reviewer completed an independent search using the same search terms on the same databases. Next, the first reviewer removed the duplicates and performed an initial screening using the title and abstract. This was checked by the second reviewer. A full text screening was then completed by the first reviewer, this was independently checked by the second reviewer. Finally, the first reviewer came to a conclusion of the selected studies, and these were independently checked and agreed to by the second reviewer. Both reviewers performed forward and backward searches of these studies by going through any sources that had cited the studies and their reference lists. No further studies were identified after this process. Any disagreements between the two reviewers were discussed and the two reviewers agreed on the final decision.

2.5. Data analysis and synthesis

Once the selected studies were finalised, key study characteristics were extracted and inserted onto a table in a Word document. Full-text screening was conducted, and the study characteristic were extracted – please refer to [Table 2](#). Critical appraisal of the studies was conducted using the Critical Appraisal Skills Programme Checklist (Critical Appraisal Skills Programme 2023) – please see [Table 3](#). The appraisal was conducted by giving each study a score according to the questions presented in the checklist, with a score of 1 for an answer of yes, 0.5 for can’t tell and 0 for no. For the last question 1 was given if the study was very valuable, 0.5 if it was moderately valuable and 0 if it wasn’t valuable. An average was determined for these scores and multiplied by 100 for a percentage; 0–20% would be classed as very poor, 21–40% as poor, 41–60% as moderate, 61–80% as good and 81–100% as excellent. These scores were split according to proportional grouping to reflect a balanced division across the scoring scale from 0–100%. The questions the studies were assessed against were related to the studies’ aims, whether the methodology was appropriate, the research design, the recruitment strategy, the data collection, the relationship between the researcher and the participant, ethical issues being considered, rigorous analysis, a clear statement of findings being available and how valuable the research is. Hazell et al. (2024), G. J. Hughes and Byrom (2019) and Spear, Morey, and van Steen (2020) scored 100%; Ramluggun et al. (2022) scored 95%; and Kendall (2017) and Payne (2022) scored 85%. The appraisal revealed that the studies included in this systematic review were of very high quality, with clear aims, high quality methodology, appropriate research design, rigorous analysis and clear findings.

In order to provide a more robust evaluation of the selected studies, a further critical appraisal was conducted using the Mixed Method’s Appraisal Tool (Hong et al. 2018) – please see [Table 4](#). The appraisal was conducted by assessing each study against the screening questions and then the questions assigned per research type. Of all the studies, five were assessed against qualitative category of study design and one

Table 1. Eligibility criteria.

	Inclusion	Exclusion
Study design and characteristics	• Studies that utilise qualitative, quantitative or mixed methods. • Studies with available full text only. • Studies from countries within the UK (England, Scotland, Wales, Northern Ireland). • Studies published in English language. • Studies published from 2014 and onwards.	• Non-UK studies. • Non-English language studies. • Studies published prior to 2014.
Population/ participants	• Studies that explore academics at British universities. • These participants can be from any university as long as it is in the UK, teach any subject and of any gender/race/ethnicity/age.	• Studies that consider the mental health literacy of academics at universities out of the UK. • Studies that consider both UK and foreign universities where UK academics views are not presented separately. • Studies that consider both primary schools, secondary schools and universities without separately presenting universities.
Exposure	• Studies that explore the mental health literacy amongst British academics. • Studies that explore how academics navigate or interact with the mental health problems that university students may present with as an indication of their mental health literacy. • Studies that explore the understanding and training British academics have had to deal with and recognise the mental health problems students present with.	• Studies that look at the support students have at universities to access mental health support without focusing on support from academics. • Studies that only discuss mental health problems in university students without considering the mental health literacy of academics.
Outcome	• Qualitative studies that explore British academics' experiences, views and recommendations about mental health literacy. • Qualitative studies that explore British university student's experiences and views in dealing with support from academics as an indication of their mental health literacy. • Quantitative studies that explore British academics' mental health literacy. • Qualitative studies that explore British university students' access to mental health support from academics as an indication of their mental health literacy. • Mixed methods studies that explore opinions and/or rates of mental health literacy amongst British academics.	• Qualitative/quantitative/mixed-method studies that do not explore the opinions, views and/or rates of mental health literacy amongst British university academics.

was assessed against the mixed methods category of study design. The questions were answered by providing a yes, no or can't tell response, and an overall comment was made about the quality of the study. The questions pertained to whether there was a clear research question, whether that particular study approach was appropriate to answer the question, the findings, interpretation, coherence, integration of different components and inconsistencies. The risk of bias in this tool was determined by the question: 'is the interpretation of results sufficiently substantiated by data?' and 'are the findings adequately derived from the data?'. Overall, this tool revealed that the

Table 2. Summary of the included studies.

Source	Study design	Participant and setting	Major themes
Hazell et al. (2024)	A qualitative design. Participants completed a questionnaire and elaborated on their answers about their relationship with their supervisor. The sample was restricted to only participants who provided qualitative data. Free text answers were analysed using reflexive thematic analysis.	Any PhD researcher at a university. 3327 PhD researchers participated in the survey and 1783 provided qualitative data. Most participants were identified as female, White British, UK citizens without any dependents.	Recognition of MHPs: - Some students feel awkward going to academics for mental health support. - The responsibility is usually down to the students to recognise their own MHPs. - Some academics are able to recognise a decline in the mental health of their students. Management of MHPs: - Some academics are not aware of what MHPs are and can worsen this for a student.
G. J. Hughes and Byrom (2019)	A qualitative approach using semi-structured interviews and focus groups. Participants were interviewed individually or in focus groups. Analysis was supported by reflection and discussion between researchers after each interview. Detailed categorisation and coding were completed independently by two researchers.	Fourteen academics teaching on professional courses were identified and interviewed. Seven of the academics participated in focus groups and seven participated in interviews.	Recognition of MHPs: - Some academics struggle recognising MHPs in students who are out of campus for placement. Management of MHPs: - Some academics struggle managing MHPs in students who are out of campus for placement. - Signposting students is difficult for some academics. Adverse effects on the academics: - Supporting students is time consuming for academics and they feel an increase in their workload. Support from universities: - Academics experience a blurring of boundaries between their role as a lecturer and a healthcare professional.
Kendall (2017)	A qualitative study through semi-structured interviews. Each interview was audio-recorded, and transcribed. A thematic analysis was conducted. Interviews were read, re-read and coded into themes.	Participants were education lecturers within one faculty in a north of England university. Twenty participants agreed to take part in the study. Ten of the participants taught on the Initial Teaching Training Programme (which focuses on training teachers), whilst the others taught on other educational programmes within the faculty.	Management of MHPs: - Some students are reluctant to approach staff for problems, in which case staff feel helpless. - Academics have varied approaches in the support they provide for students. Support from universities: - There is a lack of support from universities. - Academics would like more training.

(Continued)

Table 2. (Continued).

Source	Study design	Participant and setting	Major themes
Payne (2022)	A qualitative study. Discussions were held at a group-level. Open ended questions were derived from the literature. One two-hour staff focus groups was video recorded employing visual imagery. Thematic analysis of the interviews was conducted.	Seventeen faculty staff participated, most of which were in their 50s. Additionally, six student volunteers took part to understand their views. Most staff were in their 50s and had a career in a profession before they came into higher education or had been working in that same role for a long time.	Management of MHPs: - Academics struggle with signposting students. Adverse effects on the academics: - More students are turning to academic staff for support as counselling services are in high demand. Support from universities: - Academics are experiencing a blurring of boundaries between their pastoral and academic roles. - Academics feel as if they have no one to talk to. - Academics are unaware of the support provided to them by their university. - Academics don't think enough time is provided to manage student's MHPs.
Ramluggun et al. (2022)	A qualitative survey design that was anonymous. This was self-administered. Data were analysed using qualitative conventional content analysis.	Seventy-one faculty staff at two universities in the Southeast of England participated. Participants were faculty staff teaching on the health and social care programmes with responsibilities for student's pastoral support.	Management of MHPs: - Academics are not always confident in supporting students. Adverse effects on the academics: - Academics feel that supporting students is very time consuming and disruptive to other activities, there is a blurring of boundaries between roles. - Academics are experiencing emotional pressures from supporting students. Support from universities - Universities aren't providing clear guidelines to academics. - Current university procedures are prescriptive and inflexible. - Academics feel isolated and could benefit from opportunities where they can consult with their colleagues. - Academics would like more training.

(Continued)

Table 2. (Continued).

Source	Study design	Participant and setting	Major themes
Spear, Morey, and van Steen (2020)	A quantitative online survey, followed by qualitative semi-structured interviews. The quantitative data was analysed according to the type of institution and subject area; these variables were also considered in the qualitative data analysis. A thematic analysis was conducted on the qualitative data.	Twenty-seven institutions agreed to take part. Participants were academic staff with teaching/supervision responsibilities. One hundred and thirty academics responded to the survey. Academics from different types of institution and subject areas participated. Participants were eligible if they were academic staff with teaching/supervision responsibilities. Participants were from different types of institutions and subject areas.	Recognition of MHPs: - Staff can recognise high rates of MHPs across all institutions and subject areas. - Anxiety, stress and depression are most commonly seen. - Self-harm/suicidal ideation is also seen. Management of MHPs: - Academics are not confident in differentiating between MHPs and normal emotions. Adverse effects on the academics: - Academics fear for the safety of their students and are scared of the responsibility they hold. - Academics are experiencing disruption in their wider roles from supporting students. Support from the university: - Most academics don't feel adequately prepared by their university or trained to deal with MHPs but are still comfortable in supporting students. - Most academics would be happy to attend training.

The appraisal was conducted by giving each study a score according to the questions presented in the checklist, with a score of 1 for an answer of yes, 0.5 for can't tell and 0 for no. For the last question 1 was given if the study was very valuable, 0.5 if it was moderately valuable and 0 if it wasn't valuable. An average was determined for these scores and multiplied by 100 for a percentage; 0–20% would be classed as very poor, 21–40% as poor, 41–60% as moderate, 61–80% as good and 81–100% as excellent.

studies were of a high quality with clear research questions, appropriate study designs and data collection methods and the findings for the studies were adequately derived from the data.

To conduct a thematic analysis, the 'results' or 'findings' sections from each study were extracted and inserted into a Word document. The thematic analysis followed the methods set out by Braun and Clarke in their six-step guide (Braun and Clarke 2008). An inductive approach was used for this thematic analysis. First, the two reviewers familiarised themselves with the data. This was done by reading and re-reading the six studies to understand the contents and potential themes. The next step in the thematic analysis included initial line by line coding to generate the initial codes. This was generated by the first reviewer, and was independently checked by the second reviewer. The code was usually presented by a short phrase such as 'academics can recognise anxiety in students'. In the third step, these codes were organised by grouping them and merging them to define and name themes by the first reviewer. The fourth step involved reviewing these themes and further refining them. To ensure rigour, these themes were reviewed further and edited by the second reviewer before agreement on the final themes and sub-themes

Table 3. CASP checklist.

Question 1 –yes 0.5 – can't tell 0 – no	Hazell et al. (2024)	G. J. Hughes and Byrom (2019)	Kendall (2017)	Payne (2022)	Ramluggun et al. (2022)	Spear, Morey, and van Steen (2020)
1 – Was there a clear statement of the aims of the research?	1	1	1	1	1	1
2 – Is a qualitative methodology appropriate?	1	1	1	1	1	1
3 - Was the research design appropriate to address the aims of the research?	1	1	1	1	1	1
4 – Was the recruitment strategy appropriate to the aims of the research?	1	1	1	1	1	1
5 – Was the data collected in a way that addressed the research issue?	1	1	1	1	1	1
6 – Has the relationship between researcher and participants been adequately considered?	1	1	0	0	0.5	1
7 – Have ethical issues been taken into consideration?	1	1	0.5	1	1	1
8 – Was the data analysis sufficiently rigorous?	1	1	1	1	1	1
9 – Is there a clear statement of findings?	1	1	1	1	1	1
10 – How valuable is the research? (1 – very valuable 0.5 – moderately valuable 0 – not valuable)	1	1	1	1	1	1
Average score	100%	100%	85%	85%	95%	100%

were reached by the two reviewers. The fifth step involved finalising the themes by clearly defining them between the two reviewers, before moving onto the sixth step of producing the report.

2.6. *Synthesised findings*

The thematic analysis identified four main themes from the selected studies. This includes recognition of MHPs, management of MHPs, adverse effects on academics and support from the university. Some of these themes were similar to the themes recognised in the reviews studied but are not exactly the same or may have more or fewer themes included. The themes identified from this thematic analysis are unique to this review. By applying Bronfenbrenner’s Ecological Systems Theory, each of these themes are affected by processes occurring at different levels.

3. Results

3.1. *Included studies*

Six studies met the inclusion criteria and were included in this systematic review. Figure 1 illustrates the number of studies that were screened and excluded at each stage of the search process. A total of 10,832 studies were identified from the initial search. After initial screening of the titles, 182 results were selected. These 182 studies were assessed by reading the full text. Once this was conducted 6 studies were identified.

Table 4. Mixed methods Appraisal Tool.

Do the collected		Are the qualitative data	Are the qualitative data	Are the findings	Is the interpretation of	Is there coherence between
Are there	data allow to	approach appropriate to	collection methods	adequately derived from	results sufficiently	qualitative data sources,
clear	address the	answer the research	adequate to address the	the data?	substantiated by data?	collection, analysis and
research	research	question?	research question?			interpretation?
questions?	questions?					
MMAT Qualitative						
Hazell et al. (2024)	Yes	Yes	Yes	Yes	Yes	Yes
G. J. Hughes and Byrom (2019)	Yes	Yes	Yes	Yes	Yes	Yes
Kendall (2017)	Yes	Yes	Yes	Yes	Yes	Yes
Payne (2022)	Yes	Yes	Yes	Yes	Yes	Yes
Ramluggun et al. (2022)	Yes	Yes	Yes	Yes	Yes	Yes
MMAT Mixed Method						
Spear, Morey, and van Steen (2020)	Yes	Yes	Yes	Yes	Can't tell	Yes

Comments:
These are high-quality qualitative studies as assessed against the methodological quality criteria.
Each study scored positively against each of the factors in the assessment tool.
Comments:
This study is a high-quality mixed methods study design as assessed against the methodological quality criteria, the one criterion it lacks is that the study has not made it clear whether the divergences and inconsistencies between quantitative and qualitative data are adequately assessed.

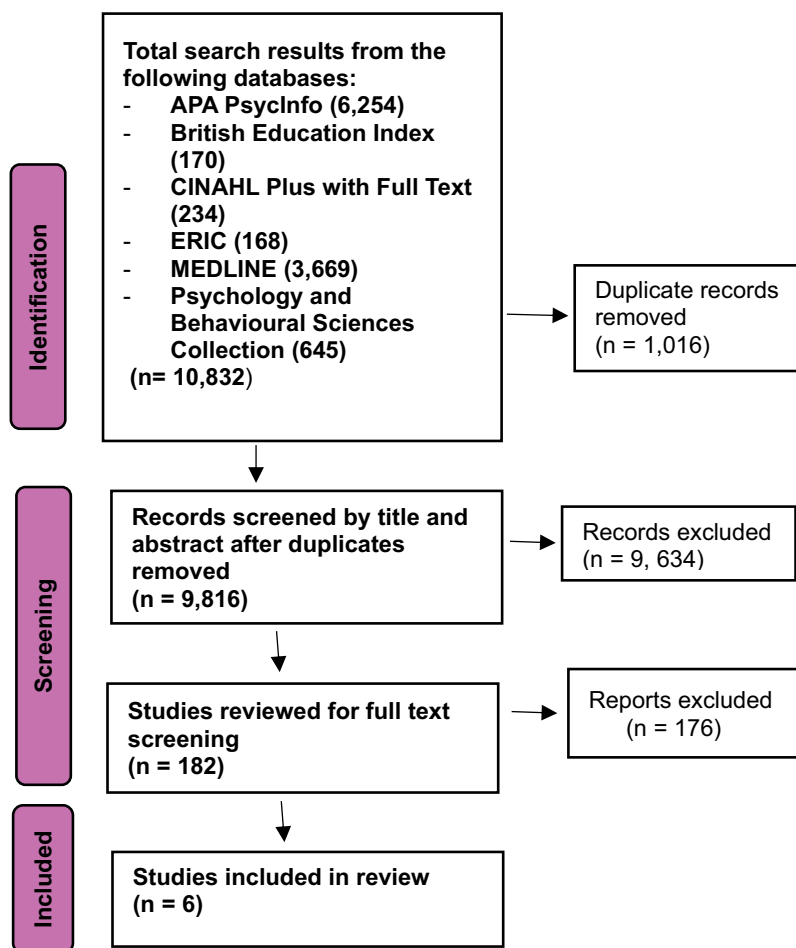


Figure 1. PRISMA flowchart for this study.

3.1.1. Recognition of mental health problems

Five studies considered whether academics can recognise MHPs in their students in some capacity (Spear, Morey, and van Steen 2020; Ramluggun et al. 2022; Payne 2022; Hazell et al. 2024; G. J. Hughes and Byrom 2019). Recognition in this context pertains to whether an academic can identify whether a student is struggling with their mental health, broadly differentiate between the common types of MHPs (e.g. depression and anxiety), and identify risk factors and triggers.

Academics can recognise that there are MHPs present in the student population, that self-harm or suicidal ideation is present and that the most common types seen are anxiety, stress and depression (Spear, Morey, and van Steen 2020).

The overwhelming issues that students are presenting with are varying degrees of depression and anxiety. (Spear, Morey, and van Steen 2020)

Students with depression and anxiety is the most common. (Spear, Morey, and van Steen 2020)

Social anxiety has been recognised as a big problem by academics:

The big issue that's emerging is students diagnosed with social anxiety. (Spear, Morey, and van Steen 2020)

Academics can also recognise certain triggers to these MHPs:

For the younger ones, the change when they get to university is often the trigger. (Spear, Morey, and van Steen 2020)

Contradictory to this, other academics have experienced problems with identifying triggers to MHPs as one states:

Some staff need help to even begin talking about mental health and spotting triggers. (Payne 2022)

Furthermore, there are some issues that have been recognised when it comes to making the distinction between normal emotions and MHPs:

There is a difference between stress and mental health problems that needs to be relayed to both staff and students. (Spear, Morey, and van Steen 2020)

Moreover, academics are found to not be sure *'if the mental health issue is significant enough to cause concern regarding their own safety'* (Ramluggun et al. 2022). Other academics can't identify certain mental health concerns:

I don't know what a low mental health concern is. (Payne 2022)

I'm not sure what early warning signs might be. (Payne 2022)

In relation to being able to identify MHPs, Hazell et al. (2024) finds that it is down to students to identify their own MHP and raise it as an issue due to the academics' inability to do so.

One student stated: 'if I wasn't explicit about my concerns and anxiety I don't know that I would receive the same support [...] They [supervisors] generally don't seem so aware or able to pre-empt them'. (Hazell et al. 2024)

Further problems in recognising MHPs have been noted in some special situations. One of these was the COVID-19 pandemic which made it hard for staff to identify students facing MHPs:

Now that things are online (in terms of academic advisor role), I find it challenging and worrying in identifying if students are struggling with their mental health – as everything is more "remote" and I miss "ad hoc"/opportunistic catch-ups with students (e.g. when face-to-face teaching and tutoring). (Ramluggun et al. 2022)

Another situation presenting difficulties with identifying MHPs in students is when students are on courses (such as healthcare courses), requiring them to go to placement and away from the university environment:

We have students isolated on placement in the sense that there will only be two or three of them working out in a hospital. (G. J. Hughes and Byrom 2019)

Overall, the recognition of MHPs by academics is affected at a microsystem with their own ability to recognise MHPs and triggers. This can also be influenced by processes

occurring at the macrosystem such as societal attitudes and how that impacts the academic's ability to recognise MHPs since more awareness amongst society will impact their own MHL. At the chronosystem level, COVID-19 has meant that remote learning has made it harder for academics to recognise MHPs (Ramluggun et al. 2022). This can be due to the fact there is less face to face interaction with students, so academics are less prone to developing relationships with their students and monitoring their mental health.

3.1.2. Management of mental health problems

All included studies discussed how academics provide support to students with MHPs. The types of support that academics were providing in the studies were identified to include talking to their students, making reasonable adjustments for them and signposting students for further specialised help. However, there are a variation of findings presented in the studies with relation to the support that students receive from academics. According to Spear, Morey, and van Steen (2020), most academics claim to be comfortable in dealing with MHPs in students – with the mean response being 2.14, where 1 = strongly agree and 5 = strongly disagree. Moreover, the following statements reveal that academics are willing to support their students.

I am happy to talk with students, be with them during difficult times, and sort out practical problems. (Spear, Morey, and van Steen 2020)

I would also like to hear the voice of the students and discuss what they would like. What would help and support them. (Kendall 2017)

Students equally discussed the support their academics provided to them:

My supervisor is aware of my anxiety and has been supportive but not pushy ... my supervisor allows me to take lead on contact and meetings, with gentle reminders if I become distant for too long. (Hazell et al. 2024)

On the other hand, there are some limitations recognised with regards to the support that academics provide. For example, academics are not always confident in supporting students with MHPs (Ramluggun et al. 2022), and some are unsure on how to make reasonable adjustments for their students (Kendall 2017). Students have also voiced wanting the academics to seem more interested in their MHPs, have more individual meetings, be more caring, empathic, listen to them more and have basic knowledge of mental health conditions (Payne 2022). One student stated about the support they received:

If I wasn't explicit about my concerns and anxiety I don't know that I would receive the same support. (Hazell et al. 2024)

Academics also spoke about the difficulties they experienced in providing support to students. On some courses, such as healthcare courses, academics feel limited with the help they can offer to students due to professional regulations placed by regulating bodies (G. J. Hughes and Byrom 2019). Moreover, in some situations where students resist speaking about their MHPs, academics found difficulties in the support that they could offer:

Whilst some students are happy to discuss how they can be helped, others won't . . . it makes it really difficult. I want to help them but what can you do? (Kendall 2017)

Other academics think that at times they cannot meet students' expectations:

They do misunderstand and overestimate sometimes the possibilities of reasonable adjustment. (Kendall 2017)

I feel that students' expectations are not being met in this sense. They are told one thing by support staff and another by the link lecturer and placement area. (Ramluggun et al. 2022)

The relationship between an academic and student has been identified as an important factor in determining the support that academics provide to students as identified by Ramluggun et al. (2022), Kendall (2017), Payne (2022) and Hazell et al. (2024). Students deem staff to be more accessible to them than counselling (Payne 2022). This is important as a good relationship with a supervisor can really help with the mental wellbeing of a student (Hazell et al. 2024), as one student states:

The relationship with my supervisor does help me a lot when I have wellbeing difficulties. (Hazell et al. 2024)

Not having a good relationship can pose difficulties as sometimes stigma may arise which can make talking about the subject of mental health difficult as one student states:

Staff should be aware of their own stigma. Some people have had it lucky, had a decent upbringing, and do not really know their body language and expressions can be put onto students without consciously realising this. (Payne 2022)

He frequently (perhaps without meaning to) creates an atmosphere of fear and intimidation [...] it also means I refuse to tell him anything about myself or my personal life or my wellbeing. (Hazell et al. 2024)

Some academics also feel as if students are reluctant to share their own MHPs because of the associated stigma (Ramluggun et al. 2022). This can cause issues because academics are then restricted to the information they have about students by being dependent on what the student wants to tell them (Ramluggun et al. 2022). This can be difficult for academics who revealed that problems arise when students do not disclose their issues during assessment times (Kendall 2017).

Academics also find difficulties in signposting students due to limitations in their understanding, as one academic said:

I was unsure what to say if I called the counselling team [on the student's behalf]. If it is suggested to the student to do this, my feeling is they may not; and [It] depends on whether [it is] a disability thing or a wellbeing thing. I would hope to put them in touch with the right people but would not really know. (Payne 2022)

A strong academic–student relationship has been identified to add further difficulty in the academic's ability to signpost:

The thing I find most difficult is that, we do have a good relationship with our students, because we teach in small classes and we cap the number of students on our course and so we always have very small cohorts in front of us, so you know them really well and one of the difficulties is, when they come to you, they don't really want you to send them somewhere else. (G. J. Hughes and Byrom 2019)

At the microsystem, academics report variation in the support they provide for students due to their own understanding of MHPs (Payne 2022) and at the mesosystem, often the academic's relationship with the student impacts this (Hazell et al. 2024). The ecosystem also impacts this as academics must stick to certain policies for specific courses which can affect the support they offer to students with MHPs. Societal attitudes and stigma which can be seen at the macrosystem level can also impact the student or the academic, thus making it harder to manage MHPs (Payne 2022). These societal attitudes and stigma can manifest as students feeling fearful to bring up their issues.

3.1.3. *Adverse effects on the academics*

The mention of adverse effects on academics because of their support to students with MHPs is identified in four studies (Spear, Morey, and van Steen 2020; Ramluggun et al. 2022; Payne 2022; G. J. Hughes and Byrom 2019). In this context, 'adverse effects' refers to any unwanted or harmful outcomes faced by the academic from providing support to students in relation to their MHPs. The two main adverse effects include: 1) the amount of time academics have to spend on students which impacts on their other roles and 2) the emotional impacts they have to deal with.

A lack of time and consequent disruption in wider roles has been identified (Spear, Morey, and van Steen 2020; Ramluggun et al. 2022; G. J. Hughes and Byrom 2019). Academics disagree that they have sufficient time to support students with MHPs and thought they should be doing other work-related activities when they were doing this (Spear, Morey, and van Steen 2020). This can be attributed to the additional administration and communication involved (Ramluggun et al. 2022). Academics claim that due to the time-consuming nature of supporting students, their workload is affected; as one academic states, they '*often have to work extra to catch up on other work*', and '*workloads increase exponentially when we need to support students' mental health*' (Ramluggun et al. 2022). Academics further reported about supporting students:

I feel (as a module leader) that we do not always have the time to look into each one [student] individually. (Ramluggun et al. 2022)

But when they drop in and they are crying, actually me saying "I need to do this marking and you haven't made an appointment" it is not helpful. So, you end up seeing them and then, yeah, every-thing else just has to wait. (G. J. Hughes and Byrom 2019)

Another stated: '*it is also incredibly time consuming and disruptive to work*' (Spear, Morey, and van Steen 2020).

The issue of academics facing an emotional toll from supporting students appears in three studies (Spear, Morey, and van Steen 2020; Ramluggun et al. 2022; Payne 2022). Academics can experience MHPs (particularly stress and worry) from dealing with the responsibility of serious MHPs present in students as evident in the following quotes from academics:

It is terrifying feeling responsible for a suicidal student and worrying if you have given the right advice. (Spear, Morey, and van Steen 2020)

If you're dealing with students who are experiencing paranoia or depression, then managing that student in the classroom ... is going to have an impact on your own mental health and sense of wellbeing and confidence. (Spear, Morey, and van Steen 2020)

Academics also mentioned how it affects them mentally beyond their contact time with the student:

Sometimes it is overwhelming, stories from some of the students that can be disturbing [...] I found some of the stories quite upsetting. Thought about it for a good while afterwards. (Ramluggun et al. 2022)

Yes it does exert an emotional strain by being concerned about their wellbeing. (Ramluggun et al. 2022)

Students also recognise the effects this was having on academics:

I think the lecturers need an outlet either as a group or privately. (Payne 2022)

Another student worried that staff '*seemed to be impacted by providing the support*' (Payne 2022).

At the microsystem level, academics may have certain personal experiences and opinions that can emotionally impact them with the support they provide for students (Ramluggun et al. 2022). The ecosystem can strongly influence the adverse effects faced by academics including the time-consuming nature of providing support and the disruption to their wider roles (Spear, Morey, and van Steen 2020). Factors such as standard workloads, contact hours, and policies can impact this. At the macrosystem, cultural attitudes such as work culture can negatively impact this as well.

3.1.4. *Support from the university*

The issue about support from universities has been identified in five studies (Spear, Morey, and van Steen 2020; Ramluggun et al. 2022; Kendall 2017; Payne 2022; G. J. Hughes and Byrom 2019). It relates to how the university supports the academics with them supporting students with MHPs. It is an important factor contributing to MHL in academics as identified in the studies. The key sub-themes pertaining to the university's support are: training from the university, university guidance and clearer definition of roles.

3.1.4.1. Training. The need to train academics to support students with MHPS is identified in four studies (Kendall 2017; Payne 2022; Ramluggun et al. 2022; Spear, Morey, and van Steen 2020). According to research by Spear, Morey, and van Steen (2020), 31% of academics do not think their institution has adequately prepared them to work with students with MHPs. Moreover, of the academics who did not receive training, 56% would attend if it was offered (Spear, Morey, and van Steen 2020). Similarly, according to Ramluggun et al. (2022), academics do not feel prepared or confident in supporting students with MHPs. Most academics claim to not have received any training in relation to supporting students with MHPs and that they would benefit from training opportunities (Ramluggun et al. 2022). Kendall (2017) also finds that every academic in their study acknowledged wanting more training in the area of mental health. Another major recommendation put forward by academics as identified by Payne (2022) is that staff want more accessible and flexible training to support students. Similar sentiments are echoed in the following statements:

Mental health first aid training or something similar would be very helpful. (Ramluggun et al. 2022)

I don't think that I have the time to embark on something in depth and lengthy, but something short and smart. It's important to know the basics in order to support your students. (Kendall 2017)

Students also recommend that staff should '*understand some examples of mental health conditions, like different types of stress or anxiety, or know the differences between stress and anxiety, to know the symptoms*'. (Payne 2022).

Due to the lack of training discussed above, students stated that some academics were ill-prepared in supporting them (Payne 2022). However, staff are still happy to offer help even if they don't feel trained as evident in the following statement by an academic:

I'm not trained, I'm not a professional, I'm not qualified, but I'll do what I can. (Spear, Morey, and van Steen 2020)

Additionally, academics have been identified to often rely on their knowledge from their professional practices rather than any professional training:

I often fall back on using previous experiences of supporting depressed patients. (Ramluggun et al. 2022)

3.1.4.2. University guidance. Four studies (Kendall 2017; Payne 2022; Ramluggun et al. 2022; Spear, Morey, and van Steen 2020) discussed the guidance provided by universities to academics in supporting students with MHPs. Overall, there is a sense of a lack of support from universities when it comes to the guidance and procedures available to academics. Spear, Morey, and van Steen (2020) finds that academics receive little guidance and as a result, '*students and staff do not know where to find help which is why some academics become very involved with supporting individuals*' (Spear, Morey, and van Steen 2020). Ramluggun et al. (2022) finds that academics would like more clarity on formal processes and procedures within the university in supporting students. They further suggest that implementing university procedures to support students are at times impractical and dependent on their job role, for example certain jobs such as being module leader means that the university procedures are too time consuming to put into practice (Ramluggun et al. 2022). Others feel that current university procedures in supporting students are prescriptive and inflexible, causing frustration by restricting academics in the support they provide or by delaying it. One academic claimed that:

It is sometimes frustrating when I would like to do more but have to abide by university rules (i.e. re exceptional circumstances when student needing extra time). (Ramluggun et al. 2022)

Academics commented on having little access to student information in order to support them, and would like information (such as individual support plans) to be shared amongst colleagues for consistency in the support the student receives (Ramluggun et al. 2022). They also state that support plans in place for students with MHPs are unrealistic and challenging to manage in practice (Ramluggun et al. 2022). Similarly, Kendall (2017) finds that academics do not feel supported by the learning support plans that are in place for students, although some do consider it to be a good resource, it lacks detail, has inaccurate recommendations and does not reflect actual requirements. The academics said about this:

They [LSPs] are very generic, especially in relation to mental health issues . . . there is nothing that helps me to support the student. They are absolutely useless; I would prefer the plans to be more specific. (Kendall 2017)

There is also a lack of reinforcement by university systems and policies in supporting students with MHPs as experienced by staff according to Payne (2022). Moreover, academics report wanting clearer guidelines on what is expected of them as one academic said in the following statement:

What is the university expecting us to do on the front line? Guidelines on how to record these encounters would be helpful. Confidentiality is about contact details for students, but not about conversations with us. Be more specific, give clearer guidance. (Payne 2022)

It is further identified that staff often report feeling not having someone to consult or speak to for support within the university. Academics mentioned feeling alone in supporting students (Ramluggun et al. 2022) and wanting someone to speak to for advice, clarity, peace of mind and to prevent isolation as evident in the following statements by academics:

I sometimes feel I need advice, someone to talk to make sure I am doing the right thing, and a "specialist"/colleague in this area would be useful to identify for supervision. (Ramluggun et al. 2022)

I think having an opportunity to talk through decisions with a colleague is helpful. (Ramluggun et al. 2022)

To speak to someone knowledgeable for support; to have someone with whom things could be thought through, clarifying the right action; and the need for some support for staff, I would say staff resilience comes into this, and wellbeing. (Payne 2022)

One academic even said with regards to having someone to go to for advice that: 'I thought it is more useful than going on a one day training course'. (Kendall 2017)

3.1.4.3. Clearer definition of roles. The issue of clear definitions of roles for academics has been identified in four studies (Spear, Morey, and van Steen 2020; Ramluggun et al. 2022; Payne 2022; G. J. Hughes and Byrom 2019). According to Ramluggun et al. (2022), academics stated that there are 'blurred boundaries' due to the multiple roles that they hold. In this study, one academic recognised this by saying '*since I, myself, am a mental health therapist I have to balance the fact that this is not my role with the students*' (Ramluggun et al. 2022). Similarly, in another study by Payne (2022), academics with additional clinical roles questioned:

Where does our professional duty stop? If I really do feel somebody is at risk, where I would still have responsibility as a nurse, so I would need to do something about that, but I wouldn't want to overstep my academic, erm, role as well. (Payne 2022)

There has also been a mention of difficulty in keeping the two roles separate:

It is difficult because we are not just academic staff, we are on a professional register as well and it is difficult to keep these two things apart and not, sort of, try to do too much in the role as personal tutor. (G. J. Hughes and Byrom 2019)

Academics identified that it is important to set boundaries, but also feel that at the same time, they are pushing their students away (Ramluggun et al. 2022). One academic added: *'I think that there needs to be very clear lines and boundaries of the types of support we should offer and when the role stops falling to us'* (Ramluggun et al. 2022).

Spear, Morey, and van Steen (2020) also demonstrated that academics had little guidance about what their role should be in supporting students. An academic in this study stated that: *'I am not a medical doctor or a social worker or a carer, it is not my job to support people with mental health issues'* (Spear, Morey, and van Steen 2020).

Overall, support from the university is influenced by processes occurring at many different levels. The mesosystem level can demonstrate how beneficial it is for academics to interact with each other as a source of support (Ramluggun et al. 2022). At the ecosystem level, the economic state of the university can affect mental health priorities, and resources available to support academics dealing with MHPS. Furthermore, at this level, there is a lack of training, guidance and funding for universities. This can be explained by processes at the macrosystem, including cultural attitudes which don't see this as a priority to be addressed.

4. Discussion

This review identified that there is a general lack of support from universities sensed by academics. A lack of training for academics was highlighted and this can be explained by the ecological systems theory. Processes such as training and guidelines at the ecosystem level are affected by those at the macro-level including societal attitudes and national campaigns. The studies in this review all discussed the need for training and clear guidance. Many academics reported not feeling prepared when it came to managing students with MHPs or not having received training to be able to do this (Spear, Morey, and van Steen 2020; Ramluggun et al. 2022; Kendall 2017; Payne 2022). Additionally, academics report feeling unclear about the limit of their involvement when supporting such students due to a lack of clarity regarding their role (Akinlotan and Wagstaff 2025). Academics also expressed being open to receiving training in this area. This is in concordance with common discourses about this issue, such as in a report published by the charity Students Minds (G. Hughes et al. 2018), who also report insufficient training for academics and an openness amongst them to receive this. The ecological systems theory demonstrates how a lack of training at the ecosystem level affects the mesosystem by affecting the relationship the academic holds with the student and at the microsystem level with the academic's own personal attitudes being influenced.

Furthermore, unclear guidance from universities is another problem faced by academics. This concern was raised during the University of Bristol v. Abrahart case (Equality and Human Rights Commission 2024), in which a university student committed suicide due to her MHPs. Subsequently, the court found that the university had 'failed to make reasonable adjustments' for the student and that 'she had been indirectly discriminated against and discriminated against on the grounds of disability' (Equality and Human Rights Commission 2024). This is highlighted as a problem as universities have an expectation from academics to support their students (Kantaris 2018). Students have also recommended that universities should do more to raise awareness of services and procedures in place to seek help (Priestley et al. 2021), which is in agreement with the

findings in this systematic review where academics often did not know where to go for help. Academics in the UK report wanting better understanding of their role and the need to establish boundaries (Kantaris 2018). The ecological systems theory explains how the absence of sufficient support from universities for academics puts them in a difficult situation when managing students with MHPs (Thompson et al. 2021). It can put academics in dangerous positions in which they may struggle to navigate appropriately managing MHPs (Douglas et al. 2023). Brewster et al. (2021) similarly claims that supporting academics is vital for them to support students.

Inadequate training and support from universities for academics can explain the adverse effects staff members experience in supporting students at the microsystem level (Payne 2022; Ramluggun et al. 2022; Spear, Morey, and van Steen 2020). This review has found that supporting students is very time consuming and disruptive to academics' wider roles and affects their own mental wellbeing. Similarly, this was found in other studies such as that by Margrove, Gustowska, and Grove (2011). Staff have also suggested they are usually de-prioritised when it comes to wellbeing and that this is likely to persist (Douglas et al. 2023). It is important to address this before the issue worsens. This can be achieved through investing in more mental health support as put forward by students in one study (Priestley et al. 2021). Studies have found that lack of training exacerbates academics' workloads (Spear, Morey, and van Steen 2020). The same reasoning can be applied for the emotional toll academics face from supporting students. With training and support from the university, there might be less anxiety among staff because they would be able to better deal with their emotions, not feel so isolated in supporting students, minimise double questioning themselves, and not be so intimidated by the responsibility of dealing with more serious MHPs as they currently are (Payne 2022, Ramluggun et al. 2022; Spear, Morey, and van Steen 2020). This study found that a whole university approach is necessary in improving MHL amongst academics. Likewise, Brewster et al. (2021) advocate for a whole university approach in promoting staff and student wellbeing.

Other consequences include that students are not receiving the support that they need (Batchelor et al. 2019). This issue is of greater importance in the current climate where universities are seeing a rise in the number of students presenting with MHPs (Sanders 2023). There is an increasing demand to meet the needs of these students (Macaskill 2012). Training and guidance would be an appropriate measure to bring in place – this has also been recognised in other research studies such as that by Sampson et al. (2022).

By applying the Consolidated Framework for Implementation Research (Damschroder et al., 2022), various contextual factors affecting the MHL of academics can be highlighted. Firstly, in terms of the intervention characteristics, the results show that interventions offered by academics differ between them and are highly dependent on their own personal expertise and experience. Some interventions offered include speaking to students, signposting them to further services and enabling reasonable adjustments for them. By exploring the outer setting, some of the external influences acting on the MHL of academics are evident. The ecological systems theory shows that this includes the support that the government offers such as the 'University Mental Health Charter Programme' and guidance regarding suicide at university (The Education Hub 2024) helping to support and promote mental health at universities. Additionally, the Office for Students has funded Student Space which acts as a mental health and wellbeing hub

(Office for Students 2025). COVID-19 was also an important external influence made evident at the chronosystem level, as the subsequent social distancing meant that universities had to resort to remote learning which studies found to be a source of worsening mental health and isolation (Lister, Seale, and Douce 2021). It also means that academics have limited access to students, making it harder to identify any MHPs (Tyler 2025). Furthermore, stigma is an important influencing factor affecting MHL in academics as it is reported that 51% of UK adults believe there is some sort of shame associated with MHPs (Mind 2024). This can hinder the support that academics can provide to students as they may feel hesitant in doing so or the student may feel ashamed in accepting this.

By exploring the inner setting, which includes the university environment, many contextual factors affecting MHL are evident. Firstly, at the macrosystem of the ecological systems theory, universities are facing significant financial cuts with the sector's net income expected to be £3.4 billion lower in 2025–26 as compared to previous deficits forecasts of £1.6 billion (Parliamentary Office of Science and Technology 2024). Additionally, many academics are facing redundancy with a reported 10,300 jobs being lost across the sector in the academic year 2023–24 (Times Higher Education 2024). The results of this review echo these statistics, as universities are unable to fund training academics, providing guidance, and enforcing mechanisms to support MHPs. Additionally, at the ecosystem level, academics are experiencing an increased workload, possibly due to taking on extra responsibilities as a result of redundancies. This review found academics can be emotionally impacted by the support that they provide for students. This can be further exacerbated or caused by the fact that academics are more likely to experience depression and anxiety (Devendorf, Wilson, and Ingram 2024), with one report finding that 53% of academics surveyed showed signs of depression (University and College Union 2024). Finally, the results from this systematic review revealed that academics are best able to support students when they have training, guidance/procedures to follow, support from other academics as well as having protected time for supporting students and their own academic work.

In order to address these barriers and facilitate change, Kotter's Change Management Model (Kotter 2007) can be applied to analyse the mental health support offered by academics at universities. Firstly, a sense of urgency already exists given that MHPs amongst university students is established as an important issue that needs to be addressed. Next, the existence of a guiding coalition could be stronger, although national and university policies exist to address MHPs amongst students, the collaboration between them and academics is minimal. Next, a stronger vision and strategy is needed; although there are national and university policies and guidelines in place, these do not offer clear plans or strategies that are known by academics or students. Simpson and Ferguson (2014) suggest that a student should be made aware multiple times and through different ways throughout their university journey in order to raise awareness about mental health support that is available. Gorczynski et al. (2017) similarly suggest making online anonymous resources available to students. The next step explores empowering others to act on the vision; unfortunately, there are many obstacles in the way of academics achieving this. These obstacles are present at a national, university and individual level. They include funding troubles (Times Higher Education 2024), workload pressures on the academic and personal emotional difficulties (Spear, Morey, and van

Steen 2020). Next, planning and creating short-term wins could look like individual universities tracking the mental health of their students through surveys before and after providing the appropriate training and guidance to academics. To consolidate and sustain improved MHL amongst academics, it is important to remove the obstacles by reconsidering mental health priorities at national and institutional levels.

4.1. Strengths

This systematic review has some strengths. Firstly, it followed the PRISMA protocol which ensures transparency and reproducibility. Secondly, the search strategy was very extensive in which many different databases were used, and an exhaustive list of search terms were utilised to ensure that as many studies could be considered as possible. The studies were also evaluated using the CASP checklist to ensure the studies' methodologies were trustworthy and relevant. This is also the first systematic review, at the time of writing this, which synthesises all primary data available exploring the MHL of academics at British universities. Furthermore, this systematic review has been conducted in a timely manner, where in the past year, the mental health of university students has been of growing concern and making headlines due to findings of a rise in MHPs (Sanders 2023).

4.2. Limitations

One limitation of this systematic review is that the search was restricted to studies published in the past 10 years. Restricting the time-period might mean there are studies published before 2014 that are excluded. This can be a limitation considering that some of these sources could have possibly provided valuable additional insight into the MHL of academics at British universities. On the other hand, these studies would not be recent. Therefore, implementing these restrictions allow for a more focused discussion and recommendation according to the UK's current context. Secondly, this systematic review had a limited number of studies that it included, with only six being eligible for use. After an exhaustive search, only six studies fit the eligibility criteria. This reflects a research gap in this subject area and therefore draws a need for more primary research to be conducted into the MHL of academics. However, the studies that were included were of high quality according to evaluation by the CASP checklist and Mixed Methods Appraisal Tool, so this systematic review has synthesised information that is trustworthy and relevant.

4.3. Conclusion

This systematic review recognises that there is insufficient training and support from universities despite the increasing role academics have in offering mental health support to students. Training, guidance and support for academics is the most important aspect to be addressed. It can be linked to the problem of inconsistent support provided to students by academics, confusion about roles and responsibilities, the disruption in wider activities and emotional problems experienced by staff. Ensuring that staff are adequately equipped with the knowledge, skills and structures by the university can

significantly improve MHL and reduce adverse effects. It is therefore vital to support academics at an institutional and national level so that they can support students.

4.4. *Recommendations*

Based on the results from this review, there are a few areas to focus on to improve MHL amongst British academics. These overarching recommendations include national policies looking after academics, increased focus and support from universities towards academics supporting students with MHPS and more research into this area.

Firstly, there are structural factors that need to be addressed for academics to feel empowered enough to support students. This systematic review found that academics are overworked with limited time to support students, this can be attributed to factors occurring at a macrosystem level. This includes the significant financial cuts in the university sector (Parliamentary Office of Science and Technology 2024) and redundancies (Times Higher Education 2024). Academics also have been reported to be more likely to experience depression (University and College Union 2024). Improving working conditions for academics by reducing staff redundancy, the subsequent immense workload, and mental health pressures they face, can allow academics to better support their students.

Universities should support academics who are dealing with students with MHPs by offering training, guidelines/policies and support from other colleagues, as this review has found that academics are not adequately trained in MHPs. Universities can implement this as mandatory training for student-facing staff. The training should inform academics how to recognise MHPs, differentiate it from normal emotions, communicate with students, develop trust with students, direct them on how to signpost and how to look after themselves. Furthermore, a recommendation put forward after the University of Bristol v. Abrahart case is relevant here and should be considered, which is that the behaviour of students should be considered in order to recognise potential MHPs. This includes their attendance, engagement with courses and activities, submission of work and discrepancies between certain modules or formats (Equality and Human Rights Commission 2024). Monitoring students' attendance and engagement can allow universities to identify worrisome changes. Simpson and Ferguson (2014) also suggest that students should be made aware multiple times about the support available for them. This can take the form of online anonymous resources as recommended by Gorczynski et al. (2017).

Furthermore, there is a need for clear guidelines that detail the boundaries of the roles of academics as mental health support systems, how involved they should be and about structures in place at universities that support academics. As recommended by the University of Bristol v. Abrahart case (Equality and Human Rights Commission 2024), a list of common reasonable adjustments should be provided for academics and other student-facing staff. Brewster et al. (2021) advocate for a whole university approach in promoting staff and student well-being. In line with this, universities can benefit from appointing an academic in each department who is adequately trained in managing MHPs in students; they should be accessible to other academics in the department who may have questions or require further guidance or just want to talk to a colleague about a student they are supporting. If there are no such structures at universities

supporting academics, this is an area of concern and should be a priority to address. Academics should feel well supported by their universities and feel that they can raise concerns or seek help both in supporting students and for their own wellbeing.

Finally, more research needs to be conducted in this area. Ideally the research should be primary and should aim to increase the number of universities, subject areas and types of higher education (i.e. undergraduate, postgraduate) that it considers. It should also aim to include the thoughts and perspective of both students and academics.

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Appendix A: Literature search strategy

Databases that were searched

- APA PsychInfo
- British Education Index
- CINAHL Plus with Full Text
- ERIC
- MEDLINE
- Psychology and Behavioural Sciences Collection

The search strategy combined the searches of:

“Mental health literacy” and “academic” and “UK” and “University” related terms

Mental health literacy terms:

- Mental health literacy
- Understanding
- Training
- Support
- Recognition
- Counselling
- Knowledge
- Insight
- Awareness
- Education
- Competence

Academic terms:

- Academic(s)
- Lecturer(s)
- Professor(s)
- University teachers(s)
- Faculty(ies)
- Educator(s)

UK terms:

- UK
- United Kingdom
- Britain
- Great Britain
- British Isles
- England
- Scotland
- Wales
- Northern Ireland

University terms:

- University(ies)
- Postgraduate(s)
- Graduate(s)
- Higher education
- College
- Master’s
- Doctoral
- Higher learning institution(s)
- Institution(s)
- Academy(ies)